AI Use Case Intake and Triage Basics

INT 200

AI use case intake is the structured process of receiving, describing, screening, and routing ideas for AI-supported work before a tool is purchased, piloted, or used. In public health, this step is essential because AI ideas often begin as informal suggestions to save time, summarize information, automate a task, or improve decision support. Without a common intake process, agencies may approve low-value or high-risk ideas too quickly while overlooking use cases that could improve timeliness, quality, equity, or workforce capacity. This module teaches learners how to describe a potential AI use case in operational terms, identify the data involved, assess the level of risk, and determine which review pathway is needed. The goal is not to turn every staff member into an AI expert. The goal is to create a practical front door so AI ideas are documented consistently and routed to the right reviewers before implementation work begins. A strong intake process protects both innovation and accountability. It helps agencies avoid ad hoc experimentation with sensitive data, unsupported vendor demonstrations, unclear ownership, and tools that do not fit public health authority or workflow needs. It also creates a record of what was considered, why it was prioritized or deferred, and what safeguards would be needed for the idea to proceed.

Level	applied/foundational
Primary track	Shared Foundational / Introductory Course
Appears in tracks	shared-foundational, governance-and-security, program-management, public-health-executive-leadership

Learning Objectives

- Describe an AI use case in plain language using the public health workflow, users, data, intended output, and decision context.
- Distinguish between low-risk, moderate-risk, and high-risk AI use cases based on data sensitivity, workflow impact, public-facing use, automation level, and potential harm.
- Identify when a use case should be routed to privacy, legal, IT, cybersecurity, equity, procurement, communications, or executive review.
- Recognize use cases that should not proceed because AI is unnecessary, inappropriate, unsafe, or unsupported by available data.
- Produce a use case intake and triage worksheet that can support governance review and portfolio prioritization.

Module Overview

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Jurisdiction and Agency Policy Note

This module provides national-level training guidance for public health agencies and partners. It is not legal advice and does not replace review by agency legal counsel, privacy officers, procurement officials, civil rights staff, records officers, or other authorized decision-makers. Requirements may vary by state, locality, territory, Tribe, agency, funding source, data-sharing agreement, emergency authority, and organizational policy. Learners should use this module to identify the issues that require review and should confirm applicable requirements before approving, procuring, deploying, or publicly communicating about AI-supported work.

Public Health Example

A communicable disease program wants to use generative AI to summarize case investigation notes and identify missing information. Intake would document the workflow, the staff who would use the output, the source data, whether protected health information is involved, whether the output would be saved in the system of record, and whether the tool would only summarize or also recommend action. Triage would likely route the use case to privacy, legal, IT, epidemiology, equity, and governance review before any pilot begins.

Topic Deep Dive

An effective intake process begins by defining the problem before selecting the technology. Staff should describe the current workflow, the pain point, the people affected, the desired improvement, and the reason AI is being considered. This prevents technology-first proposals that begin with a tool and then search for a public health problem to justify it. A clear problem statement also helps reviewers decide whether AI is needed or whether a simpler change, such as a template, dashboard, workflow redesign, or rules-based automation, would be safer and more appropriate.

The next step is to identify the data and output. Reviewers need to know whether the use case involves protected health information, personally identifiable information, confidential public health records, public documents, internal policies, vendor data, community feedback, or synthetic data. They also need to know what the AI output will be: a draft, summary, classification, prediction, recommendation, score, translation, alert, or automated action. Different outputs create different risks and require different levels of review.

Triage should evaluate both impact and readiness. Impact includes the potential effect on people, programs, public trust, legal obligations, and resource allocation. Readiness includes data quality, system integration, staff capacity, policy alignment, validation feasibility, and operational ownership. A high-impact use case with low readiness may not be rejected permanently, but it should not move into implementation until the readiness gaps are addressed.

Use case intake also supports portfolio management. When agencies collect use cases in a consistent format, leaders can compare proposals across programs, identify common infrastructure needs, avoid duplicative procurements, and align AI investments with strategic priorities. The intake record becomes the first governance artifact in the AI lifecycle.

Risks, Failure Modes, and Guardrails

Risks and failure modes include the following:

Informal AI experimentation with sensitive or confidential data before review.

Tool-first proposals that do not solve a real workflow problem.

Use cases moving forward without ownership, validation, or monitoring plans.

High-risk public-facing or decision-support uses being treated as low-risk productivity tools.

Equity, accessibility, and community impacts being considered too late.

Recommended guardrails include the following:

Require a standard intake worksheet before piloting or procuring AI tools.

Classify proposed uses by data sensitivity, automation level, public-facing impact, and decision consequence.

Route use cases to required reviewers before implementation begins.

Document use cases that are rejected, deferred, or approved with conditions.

Maintain a portfolio view of approved, active, paused, and retired AI uses.

Reflection Questions

Where does this topic appear in your agency, program, or workflow?

Which staff roles would need to understand or apply this concept before AI use is approved?

What could go wrong if this topic is not addressed before implementation?

What evidence would governance, leadership, or operations staff need before moving forward?

Which policy, data, equity, privacy, security, or workflow questions remain unresolved?

Practical Exercise

Complete a use case intake and triage worksheet for one real or realistic public health workflow. Include the problem statement, workflow, intended users, data sources, AI-supported task, output, potential benefit, risk level, required reviewers, readiness gaps, and recommendation to proceed, defer, redesign, or reject.

Expected Artifact or Evidence

Completed AI use case intake worksheet

Risk and readiness triage decision

List of required reviewers and next steps

Documented rationale for proceeding, deferring, redesigning, or rejecting the use case

Expected Assignment

- Completed AI use case intake worksheet
- Risk and readiness triage decision
- List of required reviewers and next steps
- Documented rationale for proceeding, deferring, redesigning, or rejecting the use case

Knowledge Check

- 1. What is the main purpose of AI use case intake?
- 2. Which proposal should usually receive higher-risk triage?
- 3. What should happen when a proposed workflow problem can be solved without AI?
- 4. Which item belongs in an intake worksheet?
- 5. What is strong evidence of completion?

References and Resources

- NIST Artificial Intelligence Risk Management Framework: https://www.nist.gov/itl/ai-risk-management-framework
- NIST Generative AI Profile: https://www.nist.gov/publications/artificial-intelligence-risk-management-framework-generative-artificial-intelligence
- CDC Public Health Data Strategy and Data Modernization resources: https://www.cdc.gov/public-health-data-strategy/php/about/index.html
- OWASP guidance for LLM application security
- Agency policies for privacy, cybersecurity, records, procurement, accessibility, and public communications
- Relevant public health program SOPs, data governance documentation, and implementation plans